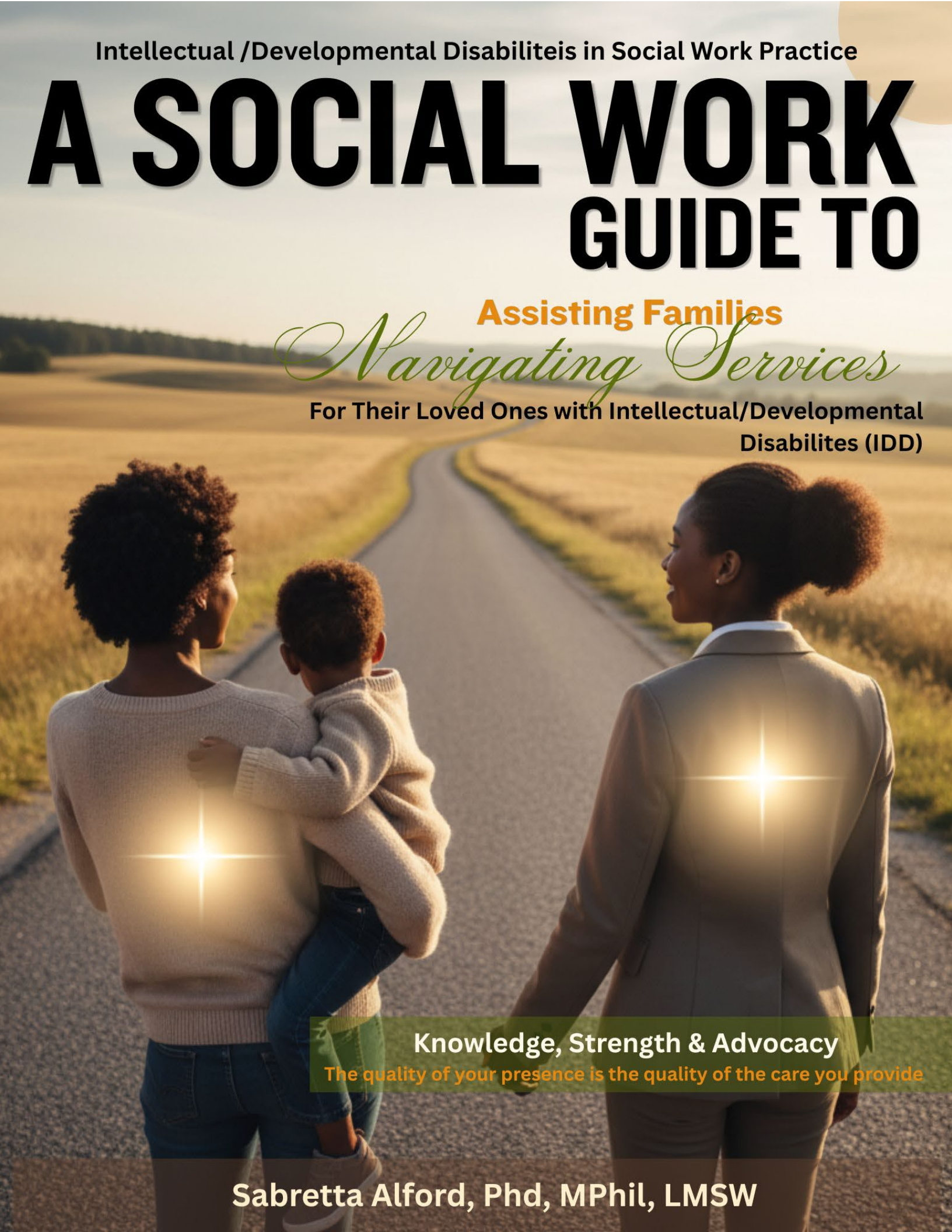
Intellectual /Developmental Disabiliteis in Social Work Practice

A SOCIAL WORK GUIDE TO

Assisting Families *Navigating Services*

For Their Loved Ones with Intellectual/Developmental Disabilites (IDD)



Knowledge, Strength & Advocacy

The quality of your presence is the quality of the care you provide

Sabretta Alford, Phd, MPhil, LMSW

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Social Work Practice Handbook

Navigating Services for Individuals with Intellectual & Developmental Disabilities (I/DD) By Sabretta Alford, PhD, MPhil, LMSW

Purpose of This Guide

Social workers are bridge-builders, connecting individuals with intellectual and developmental disabilities (I/DD) and their families to systems that are often fragmented, underfunded, and difficult to navigate.

This handbook is a practice companion for that work.

This guide draws on applied knowledge, lived experience, and system-level insight to strengthen your capacity to coordinate services, advocate effectively, and practice with ethical grounding and genuine person-centeredness.

This guide is for you.

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A Note from the Author

I want to speak directly to you, the social worker holding this handbook.

We live in a world that prizes ability. It is marketed, celebrated, and quietly demanded by all of us. But not everyone moves through life on the same terms, and that truth sits at the heart of why this work matters. Individuals with intellectual and developmental disabilities (I/DD) exist across a breathtaking range of human experience, and yet our systems, our policies, and even our profession have too often failed to meet them where they are.

I know this weight personally. I have carried it myself, and in many ways, I still do. I've also watched countless families carry it largely alone, parents and caregivers raising children with I/DD who find themselves navigating fragmented systems with little guidance, limited resources, and almost no one who understands the full picture of their lives. The research confirms what many of us already know from the inside: caregiver stress is real, it compounds over time, and the support structures meant to help are often insufficient or simply absent. This is not an abstract problem to me. It is my life, and it is the lives of the families I have dedicated my career to serving. That troubled me. It still does.

So I began asking a harder question — not just *what do these families need*, but *are we, as a profession, prepared to provide it?* The honest answer is: not yet, not consistently. Social work programs have historically underemphasized I/DD service delivery, leaving many practitioners underprepared to serve this population with the depth and confidence they deserve.

This handbook is my attempt to close some of that gap, and I mean that personally, not just professionally. It brings together what the research tells us, what years of research and practice have taught me, and what families and individuals with I/DD have shown me through their lived experience, including my own. I did not come to this work from the outside. I came to it because I had no choice, because my son needed answers that were hard to find, and because I refused to let that be the end of the story.

My goal is not to hand you a manual. It is to share what I have learned the hard way, stand beside you in this work, and offer something that is genuinely useful, born not just from scholarship ***but from life.***

You chose a profession rooted in dignity, equity, and human connection. The people described in these pages deserve all of that, and so do you.

With respect and solidarity,

Sabretta Alford, PhD, MPhil, LMSW

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Understanding the Service Delivery System

A Practical Guide for Social Workers Navigating I/DD Services in the Office for People with Developmental Disabilities

Navigating the service system for individuals with intellectual and developmental disabilities (I/DD) is complex, multi-layered, and often fragmented. Social workers are frequently the **bridge between families and systems**, making it essential to understand not just *what* the systems are, but *how they function in practice*.

Families often enter this process overwhelmed, confused, and without clear guidance. Your role is to bring clarity, structure, and direction.

> This section helps you understand the system so you can guide families effectively and confidently.

Key Systems You Will Work Within

1. OPWDD (Office for People with Developmental Disabilities)

What It Is

The Office for People with Developmental Disabilities (OPWDD) is the primary New York State agency responsible for coordinating and overseeing services for individuals with intellectual and developmental disabilities (I/DD).

OPWDD serves as the **entry point into the service system** and is responsible for determining who is eligible to receive supports and what services can be accessed.

Core Functions

- Determines eligibility for services
- Oversees the "Front Door" intake process
- Authorizes services and funding
- Contracts with provider agencies that deliver direct services
- Monitors service quality and compliance

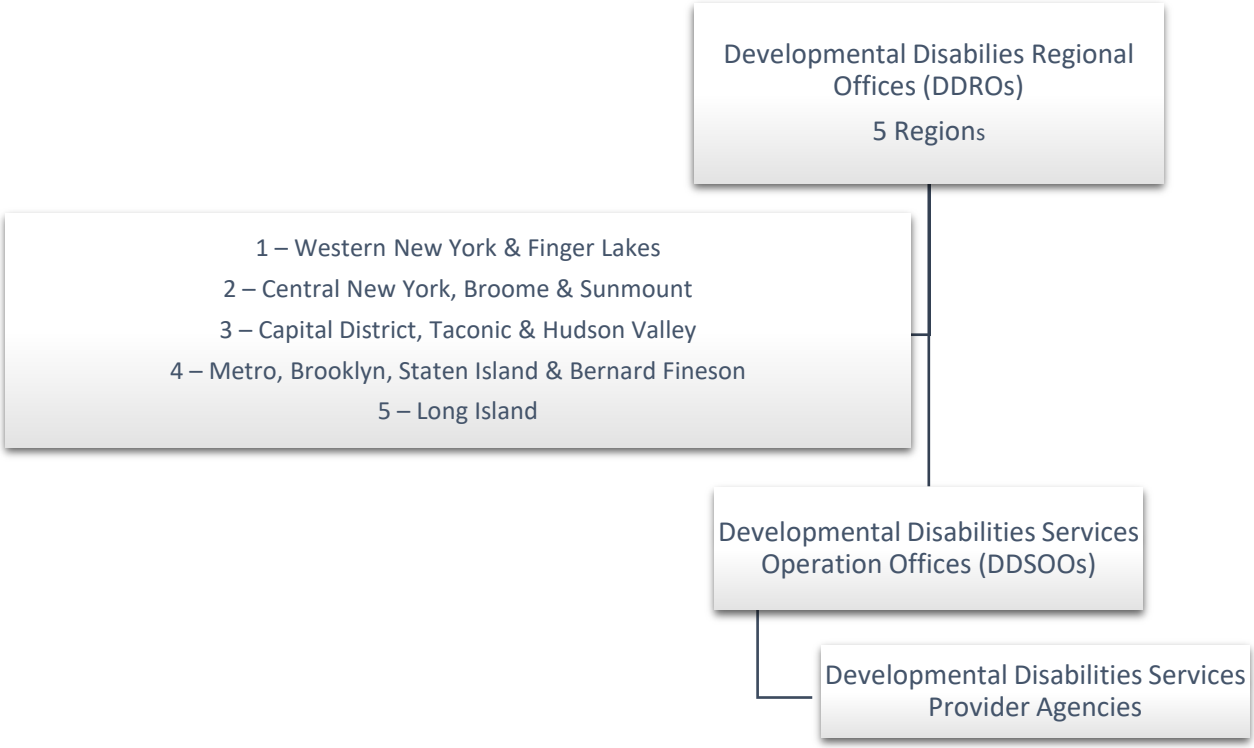
Website & Contact Information

- **Main Website:** https://opwdd.ny.gov
- **Get Started (Front Door):** https://opwdd.ny.gov/get-started
- **Front Door Process:** https://opwdd.ny.gov/get-started/front-door
- **Main Information Line:** 1-866-946-9733

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The System: The OPWDD Organization Chart

Understanding the OPWDD System



This chart shows how the OPWDD system is set up. It may look complicated, but it really comes down to ***three main parts***.

System Level	What It Does	Simple Way to Think About It
Regional Offices (DDROs)	• Review applications • Decide eligibility • Approve services	**Approval Step**
Operations Offices (DDSOOs)	• Oversee how services are organized • Help make sure services are running	**Management Step**
Provider Agencies	• Provide direct services (Com Hab, Respite, Day Programs) • Work directly with the individual	**Service Step**

How It All Works Together

1. You get approved (DDRO)
2. The system organizes services (DDSOO)
3. Services are delivered (Providers)

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What You Need to Remember as a Social Worker

- These are **separate parts of the system**
- They don't always communicate well
- Delays can happen between each step

You help:

- Keep things moving
- Explain the process to families
- Follow up when things get stuck

Takeaway: **Approval → Management → Services**

That's the system.

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DDROs – Developmental Disabilities Regional Offices

Key Contacts

These offices are critical points of contact for eligibility, application status, and system navigation:

- **Metro New York DDRO (Manhattan, Queens, Bronx):**
  646-766-3220
- **Brooklyn DDRO:**
  718-642-6000
- **Staten Island DDRO:**
  718-983-5200
- **Long Island DDRO (Nassau & Suffolk):**
  631-326-4400

What Social Workers Need to Understand (Critical Insight)

- OPWDD is a gatekeeping system—families cannot access most services without approval
- Eligibility is based on documentation, not just diagnosis
- The process can be slow, complex, and unclear to families
- Communication from the system is often inconsistent or delayed

What the "Front Door" Really Means in Practice

The "Front Door" is not just a formality; it is a multi-step intake and eligibility process that includes:

1. Registration
2. Mandatory information session
3. Submission of evaluations and documentation
4. Eligibility determination
5. Assignment to Care Management

Social workers must understand that families often do not fully grasp these steps without guidance

Practice Reality (What You Will See in the Field)

- Families unsure where to start
- Missing or incomplete documentation
- Long delays with little to no communication
- Confusion about eligibility status
- Parents feeling overwhelmed and unsupported

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Social Worker Role (Step-by-Step Support)

You are not just informing, you are **actively guiding the process**.

Your responsibilities include:

- Explaining the OPWDD process in clear, simple language
- Helping families register and begin the Front Door process
- Assisting with gathering required documentation
- Reviewing evaluations for completeness and accuracy
- Following up with DDRO offices on application status
- Preparing families for delays and next steps
- Helping families stay organized and on track

Best Practice Approach

- Do not assume the system will follow up—you must follow up
- Encourage families to keep records of all communication
- Break the process into manageable steps
- Use checklists and tracking tools
- Maintain consistent communication with families

Important Note for Social Workers

OPWDD oversees services but does not deliver them directly. Services are provided through contracted agencies, which means:

- Approval does not guarantee immediate service access
- Availability depends on providers
- Additional advocacy is often required after approval

Common Challenges Related to OPWDD

Challenge	What It Looks Like	Social Worker Action
Delayed eligibility	No response for weeks/months	Call DDRO regularly
Missing documents	Application stalled	Verify and resubmit
Lack of communication	Families left uninformed	Provide updates + advocate
Confusion about the process	Parents overwhelmed	Break into steps

Parent-Centered Practice Reminder
Families are entering this system often:

- For the first time
- Without prior knowledge
- Under stress

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Your role is to:

- Simplify the process
- Provide reassurance
- Help them move from confusion → action

Takeaway for Social Work Practice

OPWDD is the gateway to services—but not the solution on its own.

- Social workers must:
  - ☐ Guide the process
  - ☐ Monitor progress
  - ☐ Advocate consistently
  - ☐ Support families every step of the way

What Social Workers Need to Know

- OPWDD is a gatekeeping system; families cannot access most services without approval
- The process is often slow and documentation-heavy
- Families are not always guided step-by-step

Practice Reality

- Families may not receive timely updates
- Documentation requirements are strict
- Delays are common

Social Worker Role

- Explain the process in clear, simple terms
- Help families complete each step
- Track progress and follow up with DDRO offices
- Ensure required documentation is submitted correctly

1. Medicaid (Primary Funding Source)

Medicaid is the primary funding mechanism for most OPWDD services, particularly Home and Community-Based Services (HCBS).

What Social Workers Need to Know

- **No Medicaid = No Services** (in most cases)
- Medicaid eligibility is separate from OPWDD eligibility
- Delays in Medicaid = delays in services

Practice Reality

- Families often assume diagnosis alone qualifies them for services—it does not
- Applications can be delayed or denied
- Renewal and recertification issues are common

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Social Worker Role

- Assist with Medicaid applications and renewals
- Monitor application status
- Prepare families for possible delays or denials
- Support appeals if needed

2. Medicaid Managed Care

Managed Care plans coordinate healthcare services for Medicaid recipients.

What Social Workers Need to Know

- Managed Care impacts medical services, not OPWDD eligibility
- Families may need help selecting appropriate plans
- Coordination between medical and developmental services is often weak

Practice Reality

- Fragmentation between systems can lead to gaps in care
- Families may not understand how plans affect access

Social Worker Role

- Help families understand plan options
- Support coordination between providers
- Address gaps between healthcare and disability services

3. Care Management Organizations (CMOs)

Examples:

- Advance Care Alliance (ACA/NYC)
- Care Design NY

What Social Workers Need to Know

CMOs provide Care Managers who:

- Develop Life Plans
- Coordinate services
- Monitor implementation

Practice Reality

- Quality of Care Managers varies
- Communication may be inconsistent
- Families may feel unsupported

Social Worker Role

- Ensure families understand their Life Plan
- Monitor Care Manager performance
- Advocate when communication or services break down
- Support families in requesting a new Care Manager if needed

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4. Provider Agencies

What They Do:

- Deliver direct services (Com Hab, Respite, Day Programs, etc.)

What Social Workers Need to Know

- OPWDD approves services, but agencies deliver them
- Availability varies widely
- Staffing shortages are common

Practice Reality

- Families may be placed on waitlists
- Not all agencies are high quality
- Services may not match family needs

Social Worker Role

- Provide multiple provider options
- Help families vet agencies
- Monitor service quality
- Advocate for changes when needed

How the System Works Together

System	Role	Key Function
OPWDD	Gatekeeper	Eligibility & service approval
Medicaid	Funder	Pays for services
Managed Care	Healthcare	Coordinates medical care
CMO	Coordinator	Oversees service planning
Providers	Service Delivery	Direct support

Critical Insight for Social Workers

These systems do not always communicate effectively with each other.
This creates:

- Delays
- Confusion
- Service gaps

Your Role as the Connector

You are the:

- Translator of complex systems
- Coordinator across services
- Advocate when systems fail
- Support for families navigating uncertainty

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Common System Challenges (What You Will See)

- Delayed responses from agencies
- Missing or incomplete documentation
- Lack of communication between systems
- Limited provider availability
- Family overwhelm and burnout

Best Practice Approach

- Be proactive (do not wait for updates)
- Follow up consistently
- Document everything
- Educating families continuously
- Break the process into steps

Takeaway: The system is very complex, but your role is to make it manageable.

Practice Reflection

Families are not just navigating services they are navigating:

- Uncertainty
- Emotional stress
- System barriers

Your ability to **simplify, guide, and advocate** can significantly impact outcomes.

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Medicaid (Primary Funding Source)

What It Is

Medicaid is the primary public health insurance program that funds most services for individuals with intellectual and developmental disabilities (I/DD), including those authorized through OPWDD. Without Medicaid, individuals typically ***cannot access the majority of OPWDD-funded services,*** particularly Home and Community-Based Services (HCBS).

Core Functions

- Pays for OPWDD services (Com Hab, Respite, Day Programs, Residential, Self-Direction)
- Covers medical and behavioral health services
- Enables access to long-term care and supports
- Works in conjunction with SSI eligibility (in many cases)

Website & Contact Information

- **Apply / Manage Account:** https://nystateofhealth.ny.gov
- **Customer Service:** 1-855-355-5777
- **NY State Department of Health:** https://www.health.ny.gov/health_care/medicaid/

What Social Workers Need to Understand (Critical Insight)

- **Medicaid is the financial gateway to services**
- OPWDD eligibility ≠ for automatic Medicaid approval
- Delays in Medicaid directly delay service access
- Families often misunderstand eligibility requirements

Practice Reality (What You Will See)

- Families assume diagnosis alone qualifies them
- Applications delayed due to missing documentation
- Denials based on financial or technical issues
- Lapses in coverage due to missed renewals

Social Worker Role (Step-by-Step Support)

- Educate families on the importance of Medicaid
- Assist with application submission and documentation
- Ensure required documents are complete and accurate
- Monitor application status and follow up regularly
- Support renewal and recertification processes
- Assist with appeals if denied

What to Prepare (Key Documents)

- Proof of identity (birth certificate, Social Security number)
- Proof of income/resources

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- Medical and diagnostic documentation
- Proof of residence

Common Challenges

Challenge	What It Looks Like	Social Worker Action
Application delays	No updates for weeks	Follow up regularly
Denials	"Not eligible" notices	Review and appeal
Missing documents	Application stalled	Resubmit quickly
Coverage lapse	Services stop	Reinstate immediately

Best Practice Approach

- Encourage early application
- Track all submissions and confirmations
- Follow up every 7–10 days
- Prepare families for delays and appeals

Important Practice Note

If Medicaid is not active:

- Services cannot begin
- Approved plans cannot be implemented

Takeaway

Medicaid is not just insurance—it is **access to services**.

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Care Management (Service Coordination System)

What It Is

Care Management is the system responsible for coordinating services after OPWDD eligibility is approved. Families are assigned a Care Manager through a Care Management Organization (CMO).

The Care Manager is responsible for **planning, coordinating, and monitoring services**.

Core Functions

- Develops the Individualized Service Plan (Life Plan)
- Identifies needs and service goals
- Connects families to providers
- Coordinates across systems (medical, educational, social)
- Monitors service implementation

Care Management Organizations (CMOs)

Advance Care Alliance (ACA/NYC)
833-320-6589
https://acany.org

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Care Design NY
631-761-8500
https://caredesignny.org

What Social Workers Need to Understand (Essential Insight)

- Care Managers are central—but **quality varies significantly**
- Families often rely heavily on Care Managers for guidance
- Lack of communication is one of the most common complaints

Practice Reality (What You Will See)

- Families unsure of next steps
- Unreturned calls or emails
- Delays in service planning
- Incomplete or generic service plans

Social Worker Role (Critical Oversight Function)

- Ensure families understand the Life Plan
- Monitor Care Manager responsiveness
- Advocate for appropriate services
- Support families in communicating concerns
- Intervene when services are delayed or unclear

What a Care Manager Should Be Doing

- Providing regular updates
- Explaining services clearly
- Following up on referrals
- Moving services forward
- Advocating for the child's needs

➡ Red Flags to Watch For

Issue	What It Looks Like	Action
Poor communication	No response for weeks	Follow up + escalate
Lack of clarity	Family confused	Request meeting
No progress	Services not moving	Contact CMO
Dismissive behavior	Parent concerns minimized	Advocate/change CM

What to Do When There Are Problems

- Follow up directly with Care Manager
- Document all communication
- Request a meeting
- Contact the CMO
- Request a supervisor
- Request a new Care Manager

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Important Practice Reminder

- Care Managers are **partners—not replacements** for parent voice.

Takeaway for Practice

- Care Management works best when:
- Families are informed
- Social workers are involved
- Communication is consistent

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Self-Direction (Advanced Service Model)

What It Is

Self-Direction is a service model that allows families to have control over services, staffing, and budgeting, rather than relying solely on agency-managed services.
It offers flexibility—but requires active involvement.

Core Functions

- Allows families to choose staff
- Enables customized service planning
- Provides control over budget allocation
- Supports individualized care

Website & Contact Information

- **OPWDD Self-Direction Overview:**
  https://opwdd.ny.gov/types-services/self-direction

- **Training Portal:**
  https://opwdd.ny.gov/providers/training

- **Broker Support:**
  SDBroker@opwdd.ny.gov

Key Roles in Self-Direction

Role	Function
Parent	Decision-maker
Care Manager	Oversees coordination
Broker	Develops plan and budget
Fiscal Intermediary (FI)	Manages payroll and finances

What Social Workers Need to Understand (Critical Insight)

- Self-Direction offers **control + flexibility**
- It also requires **organization + responsibility**
- Not all families are prepared for this model

Practice Reality (What You Will See)

- Families are confused about budgeting
- Difficulty finding and managing staff
- Poor broker guidance
- Delays in plan approval

Social Worker Role

- Educating families on expectations and responsibilities

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- Help assess if Self-Direction is appropriate
- Monitor broker quality and performance
- Ensure budget reflects actual needs
- Support families in decision-making

The Three Core Areas of Self-Direction

- **Direct Support (Staffing)**
  - Workers providing 1:1 support
- **Purchased Services**
  - Programs, therapies, community activities
- **Administrative Services**
  - Broker + Fiscal Intermediary

Common Risks & Red Flags

Issue	What It Looks Like	Action
Poor broker support	No explanation of budget	Ask questions/change broker
Misaligned budget	Missing services	Request revision
Confusion	Parent unsure of plan	Re-educate and review
Overemphasis on admin costs	Less support for child	Rebalance budget

Best Practice Approach

- Do not rush the process
- Ensure full understanding before approval
- Encourage training participation
- Review budgets carefully
- Stay involved after implementation

Important Practice Note

- Self-Direction is not "less work"—it is:
  - More control
  - More responsibility

When Self-Direction Works Best

- Family is engaged and organized
- Strong support team is in place
- Needs are complex and individualized

Takeaway for Practice

Self-Direction is powerful—but only when:

- Families are informed
- Systems are monitored
- Social workers stay involved

Across all three systems:

Medicaid	Care Management	Self-Direction
Funding	Coordination	Customization & Control

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Intake & Eligibility: Social Worker Role

Guiding Families Through the Entry Point to Services

What This Section Is About

Intake and eligibility are the **foundation of the entire service process**. If this stage is not done correctly, everything that follows—care coordination, services, and supports—can be delayed or disrupted.

Social workers play a critical role in ensuring that families:

- Understand the process
- Submit complete documentation
- Move through the system as efficiently as possible

- **This is where your guidance can make the biggest difference.**

Core Responsibilities

Social workers are responsible for actively supporting families through the intake and eligibility process, not just providing information, but ensuring progress.

Your role includes:

- Educating families on the OPWDD "Front Door" process
- Assisting with gathering and organizing required documents
- Identifying missing or outdated evaluations
- Reviewing documentation for completeness and accuracy
- Preparing families for timelines and potential delays
- Advocating for timely review and processing
- Following up with OPWDD/DDRO when needed

Website & Contact Information

- **OPWDD Get Started:** https://opwdd.ny.gov/get-started
- **Front Door Process:** https://opwdd.ny.gov/get-started/front-door
- **OPWDD Main Line:** 1-866-946-9733
- **Regional DDRO Offices (for follow-up):**
    - Metro NY (Manhattan/Queens/Bronx): 646-766-3220
    - Brooklyn: 718-642-6000
    - Staten Island: 718-983-5200
    - Long Island: 631-326-4400

Step-by-Step: What Intake & Eligibility Looks Like in Practice

- **Initial Contact / Registration**
    - ➢ Family begins the Front Door process

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- **Information Session (Mandatory)**
  - OPWDD explains services and process
- **Document Collection & Submission**
  - Evaluations and records are gathered
- **Eligibility Review**
  - OPWDD reviews documentation
- **Eligibility Determination**
  - Approval or request for additional information

What Social Workers Must Do at Each Step

Step	Social Worker Action
Registration	Ensure family completes correctly
Info Session	Confirm attendance
Document Collection	Review for completeness
Submission	Verify all materials are included
Review Period	Follow up regularly
Determination	Prepare family for next steps

Required Documentation (Clinical Relevance)

Document	Purpose	Social Worker Role
Psychological Evaluation	Assesses cognitive functioning and diagnosis	Ensure standardized testing and scores are included
Psychosocial Evaluation	Provides family, behavioral, and environmental context	Provide or coordinate completion
Medical Diagnosis	Confirms presence of qualifying condition	Verify clarity, accuracy, and provider credentials
IEP / School Records	Demonstrates functional and developmental needs	Translate educational needs into service needs

What "Good Documentation" Looks Like

- Social workers should ensure documentation is:
- Recent and up to date
- Clearly written and detailed
- Includes standardized test scores
- Explains how the disability impacts daily functioning
- Includes specific recommendations

Practice Reality (What You Will See)

- Families submitting incomplete documents
- Outdated evaluations being rejected
- Long delays due to missing information
- Families unaware of what is required
- Confusion about why eligibility is delayed

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Practice Insight: Weak or incomplete documentation is one of the **top reasons for delays or denials**.

- This is one of the most preventable barriers—with proper social work intervention.

Common Challenges & Social Worker Actions

Challenge	What It Looks Like	Social Worker Action
Missing evaluations	Application stalled	Identify and refer immediately
Outdated reports	Not accepted by OPWDD	Request updated evaluations
Incomplete documentation	Delays in review	Review before submission
Lack of clarity in reports	OPWDD requests more info	Advocate for revisions
Family confusion	Overwhelm	Break process into steps

Best Practice Approach

- Start document collection early
- Review everything before submission
- Encourage families to keep copies (digital + printed)
- Use tracking tools (logs, checklists)
- Follow up consistently with OPWDD
- Prepare families for timelines and delays

Important Practice Notes

- Do not assume documents were received—**always confirm**
- Do not assume accuracy—**review everything carefully**
- Do not wait for updates—**follow up regularly**

Supporting Families Through This Stage

- Families may feel:
  - Overwhelmed
  - Confused
  - Frustrated by delays
  - Your role is to:
  - Provide structure
  - Simplify the process
  - Offer reassurance
  - Keep them moving forward

Practice Takeaway

- Intake and eligibility are not just administrative steps. They are the **gateway to services**.
- Strong documentation + consistent follow-up = successful outcomes.

Practice Reflection

- At this stage, social workers are not just gathering paperwork—they are:
  - Preventing delays

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    - Reducing family stress
    - Ensuring access to care
- Your attention to detail here directly impacts whether a child receives services.

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The OPWDD "Front Door" Process (System Navigation)

Guiding Families from Entry Point to Service Access

What This Section Is About

The OPWDD "Front Door" process is the mandatory entry point for accessing services. While it may appear straightforward on paper, in practice it is often complex, delayed, and unclear for families.

- Social workers play a critical role in guiding, tracking, and advocating throughout this process to ensure families do not get stuck or lost in the system.

Website & Contact Information

- **OPWDD Get Started:** https://opwdd.ny.gov/get-started
- **Front Door Process:** https://opwdd.ny.gov/get-started/front-door
- **Main Information Line:** 1-866-946-9733
- **Regional DDRO Offices:**
  - Metro NY (Manhattan/Queens/Bronx): 646-766-3220
  - Brooklyn: 718-642-6000
  - Staten Island: 718-983-5200
  - Long Island: 631-326-4400

What the Front Door Process Includes

- Registration
- Mandatory Information Session (Orientation)
- Submission of evaluations and documentation
- Eligibility review
- Eligibility determination

Social Worker Responsibilities

Social workers must actively guide families through each step, and not assume the system will do so.

Your responsibilities include:

- Guiding families through:
- Registration
- Orientation attendance
- Eligibility submission
- Ensuring all required documentation is complete and submitted correctly
- Communicating with DDRO offices regarding application status
- Tracking progress across all stages
- Following up consistently to prevent delays
- Preparing families for timelines and expectations

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Step-by-Step: Social Worker Actions

Step	Social Worker Action
Registration	Confirm submission and accuracy
Orientation	Ensure attendance is completed and recorded
Documentation	Review for completeness and quality
Submission	Verify all materials were received
Review Period	Follow up regularly with DDRO
Determination	Prepare family for next phase

➡ **System Reality (What You Will See in Practice)**

- Delays are common and expected
- Families are often not updated on status
- Communication from the system may be inconsistent
- Families may not know what step they are in
- Cases can stall without follow-up

Critical Practice Insight

- Follow-up is not optional—it is required.

✪ Cases often move forward only when someone is actively checking on them.

Best Practice Approach

- Use tracking logs for every case
- Follow up weekly (or more frequently if needed)
- Confirm receipt of all documents
- Ask specific status questions:
- "Is the application complete?"
- "Are any documents missing?"
- "What is the current timeline?"

Practice Takeaway

- The Front Door process is not passive. Therefore, Social workers must:
  - ✪ Guide
  - ✪ Track
  - ✪ Follow up
  - ✪ Advocate

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Service Planning & Care Coordination

From Eligibility to Active Services

What This Section Is About

Once eligibility is approved, the process moves into service planning and coordination**,** where services are designed and implemented.

This stage determines whether services are:

- Appropriate
- Effective
- Actually delivered

Role of the Care Manager

- Care Managers are responsible for:
- Developing the Individualized Service Plan (Life Plan)
- Identifying needs, goals, and supports
- Coordinating services across systems
- Connecting families to providers
- Monitoring service implementation

Social Worker Role (Critical Oversight Function)

Even when a Care Manager is assigned, social workers must remain actively involved.

Your role includes:

- Ensuring the family understands the service plan
- Advocating for services that reflect actual needs
- Monitoring service quality and effectiveness
- Identifying delays or breakdowns in coordination
- Addressing gaps in communication
- Supporting families in navigating the system

Practice Reality (What You Will See)

Not all Care Managers:

- Communicate consistently
- Provide clear guidance
- Follow through on tasks
- Advocate effectively
- Families may:
- Feel confused about their plan
- Experience delays in services
- Struggle to get responses

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When to Intervene

- Social workers should intervene when:
    - Communication is delayed or absent
    - Services are not moving forward
    - Plans do not reflect the child's needs
    - Families feel unsupported

Escalation Steps

1. Follow up with Care Manager
2. Document all communication
3. Request a meeting
4. Contact the Care Management Organization (CMO)
5. Speak with a supervisor
6. Request reassignment if necessary

Important Practice Reminder

- Care Managers are **partners, but not substitutes** for advocacy.

Practice Takeaway

Service planning is only effective when:

- Families understand the plan
- Services are implemented
- Systems are monitored

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Types of Services (Practice Application)

Matching Needs to Supports

What This Section Is About

Social workers must understand available services and how to match them to individual needs**.**

- **This is not a one-size-fits-all process.**

Effective service matching is critical to:
- Improving outcomes
- Supporting independence
- Reducing family stress

Core OPWDD Services

Service	Purpose	Social Work Role
Community Habilitation	Skill-building and independence	Assess functional needs and recommend supports
Respite	Caregiver relief	Identify caregiver stress and prevent burnout
Day Habilitation	Structured daily programming	Evaluate appropriateness based on age and needs
Employment Services	Job readiness and support	Promote independence and long-term goals
Residential Services	Housing and full-time support	Support long-term planning and transitions

What Social Workers Must Consider
- Developmental level
- Functional needs
- Behavioral supports required
- Family capacity and stress level
- Long-term goals (independence, employment, housing)

Practice Insight
- Service matching should always be:
- Needs-based
- Developmentally appropriate
- Family-informed

Common Mistakes in Service Matching
- Recommending services without full assessment
- Overlooking caregiver needs
- Choosing services based on availability rather than fit

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- Not adjusting services over time

Best Practice Approach

- Start with core needs
- Build services gradually
- Reassess regularly
- Collaborate with families
- Monitor outcomes and adjust

Practice Takeaway
The goal is not to assign services—

- The goal is to match services to the child's life.

Final Integration Across Sections

A. The Front Door gets families **into the system**
B. Care Management builds the **service plan**
C. Services determine the **actual quality of life**

Practice Reflection
Social workers are central at every stage:

- Entry (Front Door)
- Planning (Care Management)
- Implementation (Services)

- When social workers are actively engaged, outcomes improve.

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Self-Direction (Advanced Practice Area)

Flexible, Family-Driven Service Model

What This Section Is About

Self-Direction is one of the most flexible and individualized service models within OPWDD. It allows families to take an active role in designing and managing their child's services.

While it offers significant control and customization, it also requires ***organization, knowledge, and ongoing involvement***.

- Social workers play a critical role in helping families determine whether this model is appropriate and how to navigate it successfully.

What It Is

- Self-Direction is a service model that allows families to:
  - Control services
  - Hire and manage staff
  - Develop individualized supports
  - Manage and allocate a service budget

Rather than relying solely on agency-assigned services, families can customize supports to their child's specific needs**.**

Website & Contact Information

- **OPWDD Self-Direction Overview:** https://opwdd.ny.gov/types-services/self-direction
- **OPWDD Training Resources:** https://opwdd.ny.gov/providers/training
- **General OPWDD Contact:** 1-866-946-9733
- **Self-Direction Support (General Inquiries):** SDBroker@opwdd.ny.gov

Key Roles in Self-Direction

Role	Function
Parent / Family	Primary decision-maker
Care Manager	Oversees coordination and plan approval
Broker	Develops the Self-Direction plan and budget
Fiscal Intermediary (FI)	Manages payroll, billing, and financial administration

The Three Core Areas of Self-Direction

1. **Direct Support (Staffing):** Hiring staff to provide 1:1 support
2. **Purchased Services**: Activities, therapies, and community-based supports
3. **Administrative Services**: Broker services and Fiscal Intermediary (FI) management

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Social Worker Responsibilities

Social workers must ensure families fully understand both the ***opportunities and responsibilities*** of Self-Direction.

Your role includes:

- Educating families on:
- Responsibilities
- Risks
- Flexibility
- Helping families assess readiness for Self-Direction
- Reviewing plans and budgets for alignment with needs
- Monitoring broker performance and support quality
- Ensuring ethical use of funds
- Supporting families in decision-making and problem-solving
- Encouraging participation in training and orientation

What Social Workers Must Ensure

- The budget reflects the child's actual needs, not just available funding
- Services are appropriate and clearly defined
- Safeguards are in place for financial accountability
- Families understand how to manage staff and services

Critical Considerations

- Not all families are prepared for Self-Direction
- Brokers vary significantly in quality and responsiveness
- Families may underestimate the level of involvement required
- Oversight is necessary to ensure services are implemented correctly

Practice Reality (What You Will See)

- Families confused about budgeting and service categories
- Difficulty hiring and retaining staff
- Delays in plan approval due to incomplete submissions
- Over-reliance on brokers who may not provide adequate guidance
- Budgets that do not fully reflect the child's needs

Common Risks & Red Flags

Issue	What It Looks Like	Social Worker Action
Poor broker support	Limited explanation, lack of guidance	Recommend clarification or change broker
Misaligned budget	Missing or underfunded services	Request revisions
Parent confusion	Uncertainty about responsibilities	Re-educate and review plan
Overemphasis on administrative costs	Reduced direct support	Rebalance budget
Lack of staff	Services not implemented	Assist with recruitment strategies

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Best Practice Approach

- Do not rush families into Self-Direction
- Encourage full understanding before approval
- Review all plans carefully before submission
- Promote ongoing communication between all parties
- Start with a manageable plan and expand over time
- Monitor implementation—not just approval

Important Practice Note

Self-Direction is not a passive service model.

- It requires:
  - Active engagement
  - Ongoing oversight
  - Continuous problem-solving

When Self-Direction Is Most Appropriate

- Families are organized and engaged
- Needs are highly individualized
- Traditional services are not sufficient
- There is a strong support system in place

Practice Insight

Self-Direction requires:

- High engagement
- System knowledge
- Ongoing support

Practice Takeaway

Self-Direction offers flexibility and control but only works when:

- Families are informed
- Systems are monitored
- Social workers remain actively involved

Practice Reflection

Self-Direction can be empowering—but also overwhelming.

Your role is to:

- Clarify
- Support
- Monitor
- Protect

- When guided effectively, Self-Direction can significantly improve quality of life.

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Financial Supports (Medicaid & SSI)

Securing and Maintaining Access to Services

What This Section Is About

Financial supports are the foundation of service access for individuals with intellectual and developmental disabilities (I/DD). Programs such as Medicaid and Supplemental Security Income (SSI) are not just financial resources, they are gateways to essential services, healthcare, and long-term supports.

Social workers play a critical role in helping families:

- Understand eligibility
- Complete applications
- Maintain benefits
- Navigate challenges and appeals

- Without these supports in place, access to services can be significantly limited or delayed.

Key Financial Programs

1. Medicaid

What It Is

- Medicaid is a public health insurance program that funds the majority of services for individuals with I/DD, including OPWDD services.

What Medicaid Covers

- Medical care (doctor visits, hospital care)
- Behavioral health services
- Therapies and supports
- Home and Community-Based Services (HCBS)
- Long-term care and residential supports

Website & Contact Information

- **Apply / Manage Account:** https://nystateofhealth.ny.gov
- **Customer Service:** 1-855-355-5777
- **NY State Medicaid Information:** https://www.health.ny.gov/health_care/medicaid/

Critical Insight for Social Workers

- Medicaid is often required before services can begin
- Eligibility is based on both financial and disability criteria
- Delays in Medicaid approval delay all other services

2. Supplemental Security Income (SSI)

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What It Is SSI

- SSI is a federal program that provides monthly financial assistance to individuals with disabilities who meet income and resource requirements.

What SSI Provides

- Monthly income support
- Automatic or streamlined access to Medicaid (in many cases)
- Financial stability for families

Website & Contact Information

- Social Security Administration (SSA): **https://www.ssa.gov**
- SSI Information: **https://www.ssa.gov/ssi**
- Main Phone Number: **1-800-772-1213**

Critical Insight for Social Workers

- SSI approval can help facilitate Medicaid eligibility
- Families often rely on SSI for basic financial support
- Application processes can be lengthy and complex

Social Worker Role

Social workers must take an active role in ensuring families successfully navigate financial support systems.

Your responsibilities include:

- Assisting with applications for Medicaid and SSI
- Explaining eligibility requirements in clear terms
- Ensuring all required documentation is complete
- Monitoring application status and timelines
- Supporting families through renewal and recertification processes
- Assisting with appeals if applications are denied
- Educating families on maintaining benefits

Step-by-Step: Supporting Families Through the SSI Process

Step	Social Worker Action
Application	Assist with completion and submission
Documentation	Ensure accuracy and completeness
Submission	Confirm receipt of application
Review Period	Monitor status and follow up
Decision	Prepare family for approval or denial
Post-Approval	Support maintenance and renewal

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➡ **Common Issues (What You Will See in Practice)**

- Denials due to incomplete or unclear documentation
- Delays in application processing
- Coverage interruptions due to missed renewals
- Families unaware of eligibility requirements
- Confusion about differences between programs

Common Challenges & Social Worker Actions

Challenge	What It Looks Like	Social Worker Action
Denial	"Not eligible" notice	Review reason and initiate appeal
Delay	No update for weeks/months	Follow up regularly
Missing documents	Application stalled	Resubmit immediately
Coverage lapse	Services stop	Reinstate benefits quickly
Renewal issues	Missed deadlines	Track and remind families

Best Practice Approach

- Begin applications as early as possible
- Keep copies of all submitted documents
- Track application dates and deadlines
- Follow up consistently (do not wait for updates)
- Prepare families for possible delays and outcomes
- Always review denial letters carefully

Appeals: What Social Workers Must Do

- If a denial occurs:
  - Review the denial reason carefully
  - Identify missing or incorrect information
  - Gather additional documentation
  - Submit an appeal within the required timeframe
  - Follow up consistently until resolved

Important Practice Note

Many denials are not final—they are often due to:

- Missing documentation
- Incomplete applications
- Technical errors

✪ Appeals can and should be pursued when appropriate.

Practice Reminder

Always prepare families for the possibility of appeals.
This helps:

- Reduce frustration

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- Set realistic expectations
- Encourage persistence

➡ **Supporting Families Emotionally**

- Families navigating financial systems may feel:
  - Stressed
  - Confused
  - Frustrated by delays
  - Your role is to:
  - Provide clarity
  - Offer reassurance
  - Keep them informed
  - Encourage persistence

Practice Takeaway

Financial support is not just a benefit—they are access point to care and stability.

Practice Reflection

Your role in this process directly impacts whether families:

- Gain access to services
- Maintain stability
- Avoid unnecessary delays

✪ Advocacy in financial systems is essential to effective social work practice.

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Advocacy & Ethical Practice

Centering Families, Challenging Systems, and Upholding Ethical Care

What This Section Is About

Advocacy and ethical practice are not separate from service delivery—they are central to effective social work with individuals with intellectual and developmental disabilities (I/DD) and their families.

Social workers are often positioned at the intersection of:

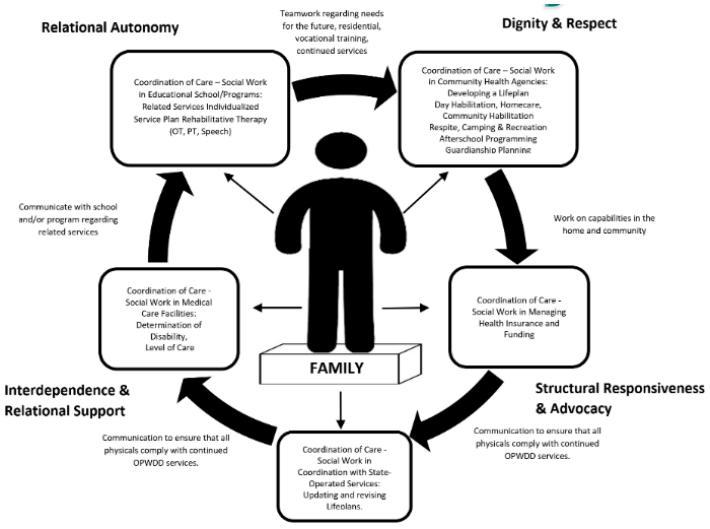
- Families
- Service systems
- Policy structures
- This requires the ability to:
- Navigate systems
- Uphold ethical standards

Actively advocate for equitable access and treatment

✪ This section grounds practice in the **Person-Centered Ethics of Care (PC-EoC) Model**, ensuring that social workers remain aligned with both ethical responsibility and family-centered practice.

The **Person-Centered Ethics of Care (PC-EoC) Model** was developed by Sabretta Alford and Esther Son (2025) as a disability-informed framework for social work practice. It integrates person-centered principles with core ethical domain-relational autonomy, dignity and respect, interdependence, and structural responsiveness, to guide practitioners in delivering ethical, family-centered, and systems-aware care.

Core Principles (Aligned with PC-EoC Model)



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1. Relational Autonomy → Respect Family Expertise
Families are not passive recipients of services—they are **co-experts** in their child's life.

What This Means in Practice:

- Recognize parents as primary knowledge holders
- Include families in all decision-making processes
- Avoid dismissing concerns or lived experience
- Support shared decision-making

2. Dignity & Respect → Honor Lived Experience
Families navigating disability systems often face emotional, social, and systemic challenges.

What This Means in Practice:

- Treat families with respect at all times
- Validate their experiences and concerns
- Avoid judgment or deficit-based language
- Recognize the emotional labor of caregiving

3. Interdependence → Collaborate Across Systems
No single system meets all needs. Effective care requires collaboration across multiple systems**.**

What This Means in Practice:

- Coordinate across OPWDD, Medicaid, schools, and providers
- Facilitate communication between systems
- Break down silos that create service gaps
- Promote team-based approaches

4. Structural Responsiveness → Address Systemic Barriers
Many barriers families face are **systemic—not individual**.

What This Means in Practice:

- Identify patterns of inequity
- Advocate for system-level changes
- Address delays, access issues, and policy limitations
- Move beyond individual solutions to structural awareness

Social Worker Advocacy Role
Advocacy is not optional—it is a **core function of social work practice**.

Key Responsibilities

- Amplify parent voice in all service planning and decision-making
- Challenge system inefficiencies that delay or limit services
- Prevent service gaps through proactive monitoring and follow-up
- Address inequities in access, quality, and delivery of services

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- Ensure services reflect the child's actual needs—not system limitations
- Advocate within and across systems (OPWDD, Medicaid, providers)

Advocacy in Action (What This Looks Like)

- Following up when services are delayed
- Questioning decisions that do not align with needs
- Escalating concerns when communication breaks down
- Supporting families in expressing their concerns clearly
- Ensuring plans are individualized—not generic

Practice Reality (What You Will See)

- **Parents navigating systems are often:**
  - Misunderstood
  - Labeled as "difficult" or "non-compliant"
  - Viewed as challenging when advocating for their child
  - Excluded from decision-making processes
  - Expected to accept limited or inadequate services

Practice Insight

What is often labeled as "difficult behavior" is actually:

- Advocacy
- Persistence
- Protection of their child

Social Worker Responsibility: Countering System Dynamics

- Social workers must actively work to shift these dynamics.

What This Requires

- Listening without defensiveness
- Validating parent concerns
- Reframing advocacy as a strength
- Ensuring parents are included in all decisions
- Challenging dismissive or biased responses from systems
- Supporting families in navigating power imbalances

When to Intervene

- When parent concerns are minimized
- When services do not reflect needs
- When communication breaks down
- When systems delay or deny access without a clear reason

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Common Ethical Challenges

Challenge	What It Looks Like	Social Worker Action
Power imbalance	Professionals override parents	Re-center parent voice
System bias	Families labeled negatively	Reframe and advocate
Service limitations	"This is all we offer"	Push for alternatives
Lack of transparency	Families uninformed	Provide clarity

Best Practice Approach

- Center the family in all decisions
- Communicate clearly and consistently
- Document concerns and actions
- Advocate early—do not wait for escalation
- Maintain ethical accountability at all times
- Recognize advocacy as part of your professional role

Important Practice Note

- Advocacy does not mean conflict—it means:
  - Ensuring fairness
  - Promoting access
  - Protecting rights
  - Supporting dignity

Practice Takeaway

- Ethical practice in this work requires:
  - Listening
  - Respecting
  - Advocating
  - Acting

Practice Reflection

Social workers are uniquely positioned to either:

Reinforce system barriers
or
Disrupt them

✪ The difference lies in your willingness to advocate.

Final Thought

- When systems fail families, advocacy becomes essential—not optional.
- Your role is not just to navigate the system
- It is to improve how the system responds to families.

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Common System Barriers (Professional Perspective)

Recognizing System Breakdowns and Responding Effectively

What This Section Is About

Social workers navigating systems for individuals with intellectual and developmental disabilities (I/DD) will consistently encounter systemic barriers that impact access to services, quality of care, and overall outcomes.

These barriers are not isolated; they are often structural, recurring, and predictable**.**

Understanding these challenges allows social workers to:

- Respond proactively
- Support families more effectively
- Reduce delays and frustration
- Advocate strategically

✪ This section prepares you to recognize barriers and respond with action.

Key System Barriers and Social Worker Responses

Barrier	System Issue	Social Worker Action
Communication delays	System overload, high caseloads, limited responsiveness	Follow up consistently, document all contact, escalate when needed
Provider shortages	Workforce crisis, staffing limitations	Expand provider search, explore alternative services, maintain multiple options
Eligibility delays	Bureaucratic processes, documentation requirements	Monitor status, follow up with DDRO, escalate if timelines are exceeded
Poor care management	Inconsistent training, high caseloads, variable quality	Advocate for improved communication, involve CMO, request reassignment if needed
Parent burnout	System strain, prolonged stress, lack of support	Provide emotional support, connect to respite and resources, validate experience

Understanding the System Behind the Barriers

These challenges are often rooted in:

- High system demand and limited resources
- Complex administrative processes
- Fragmentation across systems (OPWDD, Medicaid, providers)
- Workforce shortages in direct care roles
- Lack of consistent communication protocols

✪ These are system-level issues—not individual failures

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➡ **Practice Reality (What You Will See)**

- Calls and emails going unanswered
- Families placed on long waitlists
- Applications delayed without explanation
- Care Managers inconsistently engaged
- Families becoming overwhelmed and exhausted

Social Worker Role: Moving from Reaction to Action

Social workers must shift from:

- Waiting → Following up
- Accepting delays → Questioning timelines
- Isolated action → Coordinated response

Barrier-Specific Practice Strategies

1. Communication Delays

What It Looks Like:

- No response from agencies or providers
- Delayed updates on applications or services

What to Do:

- Follow up weekly (or more frequently if urgent)
- Keep a detailed communication log (date, contact, outcome)
- Send follow-up emails after phone calls
- Escalate when communication remains unresponsive

2. Provider Shortages

What It Looks Like:

- "No staff available"
- Long waitlists for services

What to Do:

- Provide families with multiple agency options
- Encourage direct outreach to providers
- Expand search geographically if possible
- Reassess service types if needed

3. Eligibility Delays

What It Looks Like:

- Applications stalled for weeks or months
- No updates from OPWDD

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What to Do:

- Contact DDRO offices regularly
- Confirm all documents were received
- Ask for clear timelines
- Escalate when delays exceed expected timeframes

4. Poor Care Management

What It Looks Like:

- Lack of communication
- Delayed service coordination
- Families unsure of next steps

What to Do:

- Request updates from Care Manager
- Schedule case meetings
- Contact the Care Management Organization (CMO)
- Request reassignment if necessary

5. Parent Burnout

What It Looks Like:

- Emotional exhaustion
- Frustration with the system
- Reduced ability to follow through

What to Do:

- Normalize the experience
- Connect families to respite services
- Encourage use of support networks
- Provide reassurance and structure

Best Practice Approach

- Anticipate barriers before they occur
- Communicate clearly and consistently
- Document all actions and outcomes
- Provide families with realistic expectations
- Offer both practical and emotional support

Important Practice Reminder

Barriers are not a sign that the system has failed completely

- ✪ They are a sign that **intervention is needed**

Practice Takeaway

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Barriers are part of the system.

- Your role is to:
  - Identify them
  - Respond to them
  - Work around them
  - Advocate through them

Practice Reflection
How social workers respond to barriers determines whether families:

Remain stuck
or
Move forward

Final Thought
<u>Families often experience these barriers alone.</u>

✪ Your presence, persistence, and advocacy can transform that experience.

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Trauma-Informed & Family-Centered Practice

Supporting Families with Awareness, Compassion, and Structure

What This Section Is About

Families navigating systems for individuals with intellectual and developmental disabilities (I/DD) are often experiencing **ongoing, cumulative stress**. This is not a one-time event—it is a continuous process that can impact emotional well-being, decision-making, and engagement with services.

Trauma-informed and family-centered practice requires social workers to understand not only the system but also the emotional impact of navigating it**.**

- This approach shifts practice from "What is wrong?" to "What has this family experienced—and what do they need now?"

What Families Are Experiencing

- Families navigating these systems often experience:
- Chronic stress from ongoing responsibilities and uncertainty
- Emotional fatigue from repeated barriers and delays
- System distrust due to negative past experiences
- Frustration with lack of communication or progress
- Feelings of isolation and lack of support

Understanding the Impact

- These experiences can lead to:
- Difficulty following through with tasks
- Increased anxiety during decision-making
- Heightened emotional responses
- Reduced trust in providers and systems

✪ These are not signs of resistance—they are responses to prolonged stress.

Social Worker Responsibilities

Social workers must intentionally integrate trauma-informed and family-centered principles into their daily practice.

Core Responsibilities:

- Normalize family experiences and emotional responses
- Provide clear, step-by-step guidance
- Reduce overwhelm by breaking down processes
- Support caregiver well-being alongside service planning
- Create a sense of safety and trust in interactions
- Communicate consistently and transparently

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What This Looks Like in Practice

- Explaining processes in simple, manageable steps
- Checking in regularly with families, not just when there is an update
- Acknowledging frustration without dismissing it
- Allowing space for questions and repeated clarification
- Offering reassurance when systems feel overwhelming

Common Practice Challenges

Challenge	What It Looks Like	Social Worker Response
Overwhelm	Family unsure where to start	Break process into steps
Frustration	Emotional responses to delays	Validate and guide
Withdrawal	Reduced communication	Re-engage with support
Distrust	Hesitation to follow guidance	Build consistency and transparency

Best Practice Approach

- Use clear, simple language
- Avoid overwhelming families with too much information at once
- Provide written follow-up when possible
- Check for understanding (not just agreement)
- Be consistent and reliable in communication

Important Practice Reminder

- Families are not "non-compliant"—they are often overwhelmed.

Reframing Behavior in Practice

Instead of asking:
"Why aren't they following through?"
Ask:
"What barriers are they facing?"
"What support do they need to move forward?"

Supporting Caregiver Well-Being

- **Social workers should also:**
  - Encourage use of respite services
  - Connect families to support groups or networks
  - Acknowledge caregiver stress and emotional labor
  - Reinforce that seeking help is appropriate and necessary

Practice Insight

Caregiver well-being directly impacts:

- Service engagement

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- Decision-making
- Family stability
- Child outcomes

Practice Takeaway

- Trauma-informed practice is not an additional task—it is ***how the work should be done***.

Practice Reflection

- Families remember how they were treated—not just what services they received.

Your approach can either:

Reduce stress
or
Add to it

When social workers lead with empathy, clarity, and consistency:

- Families feel supported
- Systems become more manageable
- Outcomes improve

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Supporting Parent Well-Being

Sustaining Families to Sustain Services

What This Section Is About

Supporting parent well-being is not separate from service delivery—it is **essential to it**. Families caring for individuals with intellectual and developmental disabilities (I/DD) often carry long-term emotional, physical, and logistical responsibilities that can lead to burnout over time.

When caregivers are overwhelmed, it directly affects their ability to:

- Navigate systems
- Follow through with services
- Make informed decisions
- Maintain stability within the home

Social workers must intentionally assess and support caregiver well-being as part of effective practice.

Understanding Caregiver Experience

- Parents and caregivers may experience:
- Chronic stress from ongoing responsibilities
- Emotional fatigue from repeated system challenges
- Isolation due to limited support networks
- Frustration with delays and lack of progress
- Guilt when prioritizing their own needs

Social Worker Role

Supporting caregiver well-being requires proactive, consistent engagement.

- Your responsibilities include:
- Assessing signs of burnout and emotional strain
- Encouraging the use of respite services when available
- Connecting families to peer support and community resources
- Validating emotional experiences without minimizing them
- Helping families create realistic expectations and boundaries
- Providing structure and guidance to reduce overwhelm

What Burnout May Look Like

Indicator	What It Looks Like
Emotional exhaustion	Constant fatigue, irritability, withdrawal
Reduced follow-through	Missed appointments, delays in paperwork
Decision fatigue	Difficulty making choices or planning
Frustration with systems	Increased stress during interactions
Isolation	Limited engagement with supports

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Critical Insight
Burnout directly impacts:

- Service follow-through
- Decision-making
- Family stability

When caregiver well-being declines, service outcomes are affected as well.

Why This Matters in Practice

- Caregivers are often responsible for:
- Coordinating services
- Managing appointments
- Communicating with providers
- Advocating across systems

When overwhelmed, even highly engaged caregivers may struggle to maintain these responsibilities.

Best Practice Approach

- Regularly check in on caregiver well-being—not just service status
- Normalize the challenges of caregiving
- Encourage small, manageable self-care practices
- Promote use of available supports (respite, community programs)
- Provide consistent and predictable communication

Supporting Through Conversation

Social workers should:

Ask open-ended questions:
"How are you managing everything right now?"
"What support would feel most helpful at this moment?"
Validate responses:
"It makes sense that this feels overwhelming."
"You've been handling a lot."

Connecting Families to Support

Encourage access to:

- Respite services
- Parent support groups
- Community-based organizations
- Informal support networks (family, friends)

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Important Practice Reminder

- Caregiver well-being is not a secondary concern—

It is central to sustained engagement and effective service delivery.

Practice Takeaway

When caregivers are supported:

- They engage more effectively
- Services are more consistent
- Outcomes improve

Practice Reflection

Supporting the child requires supporting the caregiver.

- When caregivers are overwhelmed, systems feel harder to navigate.
- When caregivers are supported, systems become more manageable.

You are not only supporting services—you are supporting the people who make those services possible.

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Caregiver Burnout Assessment Tool

For Social Workers Supporting Families of Individuals with I/DD

Rate each item from 0 (Not at all) to 3 (Almost always).

Domain	Item	Score (0-3)
Emotional Well-Being	Feels emotionally exhausted	
	Feels overwhelmed by responsibilities	
	Experiences frustration or irritability	
	Feels unsupported or alone	
Physical & Mental Fatigue	Reports constant tiredness	
	Has difficulty sleeping or resting	
	Feels physically drained	
	Has limited time for personal needs	
Caregiving Strain	Feels caregiving is too much	
	Struggles to manage daily responsibilities	
	Feels there is no break from caregiving	
	Difficulty balancing caregiving with other roles	
System Navigation Stress	Frustrated with service systems	
	Difficulty keeping up with requirements	
	Confused about next steps	
	Distrust in providers or systems	
Impact on Functioning	Misses appointments or deadlines	
	Difficulty making decisions	
	Withdraws from communication	
	Unable to keep up with services	
	Total Score:	

Scoring Guide

0 – 10 — Low Minimal burnout indicators	**11 – 25 — Moderate** Emerging signs; monitor closely	**26 – 45 — High** Significant burnout; support needed	**46+ — Severe** Crisis level; immediate intervention

Completed by: _____ Date: _____ Case #: _____

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Important Practice Note: Burnout often develops gradually, making escalation less likely.

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Quick Burnout Screening Checklist

For Fast Use in Practice (2-3 Minutes)

Ask or Observe the Following:	
□ **Does** the caregiver appear overwhelmed?	□ **Are** they having difficulty making decisions?
□ **Are** they expressing frustration or exhaustion?	□ **Do** they report little or no personal time?
□ **Have** they missed appointments or deadlines?	□ **Are** they withdrawing from communication?
□ **Do** they seem unsure about next steps?	□ **Do** they express distrust or frustration with systems?
	Total Checked: _____

Quick Interpretation

0 - 2 checked	3 - 5 checked	6+ checked
Likely Managing	**At Risk**	**High Concern**
Monitor and continue check-ins	Provide support and resources	Intervene immediately

Clinician: _____ **Date:** _____ **Case #:** _____

Notes / Observations:

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Immediate Response Guide

When caregiver burnout concerns are identified

If concerns are identified, take these steps:

Step 1

Acknowledge & Validate

"You've been managing a lot—it makes sense this feels overwhelming."

Let caregivers feel heard before offering solutions.

Step 2

Provide Structure

Break tasks into smaller, manageable steps:

- Prioritize the most urgent needs first
- Create a simple weekly action plan
- Set realistic and achievable goals

Step 3

Offer Support

Connect caregivers to:

- Respite services
- Peer support groups
- Follow-up check-ins
- Community and family networks

Best Practice Tip

Do not wait for caregivers to ask for help

- Many will not ask—proactive outreach saves cases.

Watch for:

- Missed appointments or reduced contact
- Shorter or more frustrated responses
- Signs of withdrawal or disengagement

Simple Takeaway

Burnout affects everything.

- When caregivers are supported, services are more effective.

Remember:

- Caregiver well-being = child well-being
- Early support prevents crisis
- Consistency builds trust over time

Respond → Acknowledge → Structure → Support → Follow Up

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Red Flags in Practice

Social workers should be alert to the following concerns

Red Flag 1

Lack of Service Implementation

- Services are not being delivered as planned
- Caregiver reports provider no-shows or gaps
- No evidence of service activity in records

Action: Contact provider immediately and document

Red Flag 2

Poor Communication from Providers

- Unanswered calls, emails, or messages
- Inconsistent or incomplete progress updates
- Provider unable to confirm service delivery

Action: Escalate to supervisor and log all attempts

Red Flag 3

Misaligned Service Plans

- Services do not reflect individual's current needs
- Plan not updated following assessments or changes
- Goals are unclear, outdated, or contradictory

Action: Request immediate plan review and revision

Red Flag 4

Financial Misuse in Self-Direction

- Funds used for non-approved purposes
- Missing or inconsistent spending documentation
- Caregiver unaware of budget or expenditures

Action: Report to fiscal intermediary and supervisor

⚠ ACTION REQUIRED: Address immediately—do not delay.

Best Practice Tip

When a red flag is identified:

- Do not wait to see if it resolves on its own
- Document findings immediately and thoroughly
- Consult your supervisor before taking action alone
- Follow up in writing to create a paper trail

Quick Reference Rule

If you see a red flag, always ask:

- Is the individual safe and receiving services?
- Has this been documented in the case record?
- Has the supervisor been notified?
- What is the next action and timeline?

Identify → Document → Notify → Act → Follow Up

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Best Practices for Social Workers

Delivering Effective, Ethical, and Systems-Responsive Practice

What This Section Is About
Working within systems that serve individuals with intellectual and developmental disabilities (I/DD) requires more than knowledge—it requires intentional, consistent, and strategic practice.

Best practices are not just recommendations—they are the difference between:

- Delayed vs. timely services
- Confusion vs. clarity
- Frustration vs. progress

This section outlines the core approaches that support effective outcomes for families.

Core Best Practices

Be Proactive, Not Reactive
Social workers must anticipate challenges before they become barriers.

- What This Looks Like in Practice:
- Following up before deadlines are missed
- Identifying potential delays early
- Preparing families for each step in advance
- Monitoring progress without waiting for problems

Communicate Clearly and Consistently
Clear communication reduces confusion, builds trust, and keeps families engaged.

What This Looks Like in Practice:

- Using simple, accessible language
- Providing regular updates—even when there is no new information
- Confirming understanding (not just agreement)
- Following up conversations with written summaries when possible

Document Everything
Documentation protects both the family and the social worker while ensuring continuity of care.

What This Looks Like in Practice:

- Recording all communication (calls, emails, meetings)
- Tracking dates, contacts, and outcomes
- Keeping organized records of applications and submissions
- Documenting concerns and actions taken

Educate Families Continuously
Families should not have to navigate systems alone or without understanding.

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What This Looks Like in Practice:

- Explaining each step of the process
- Repeating information as needed
- Providing tools, guides, and resources
- Encouraging questions and clarification

Advocate Across Systems
Social workers must move beyond single-system thinking and advocate within and across multiple systems.

What This Looks Like in Practice:

- Coordinating between OPWDD, Medicaid, schools, and providers
- Challenging delays or gaps in services
- Escalating concerns when necessary
- Ensuring services reflect actual needs

Stay Solution-Focused
Systems can be complex and frustrating—but effective practice requires maintaining a focus on solutions.

What This Looks Like in Practice:

- Identifying next steps, even when barriers exist
- Offering alternatives when services are unavailable
- Breaking down problems into manageable actions
- Maintaining forward movement

Common Pitfalls to Avoid

Pitfall	Impact	Best Practice Alternative
Waiting for systems to respond	Delays in services	Follow up consistently
Using technical language	Family confusion	Use clear, simple language
Incomplete documentation	Gaps in care	Document thoroughly
Providing one-time explanations	Misunderstanding	Reinforce and repeat
Accepting system limitations	Reduced services	Advocate for alternatives

Best Practice Integration
Effective social work practice in this context requires:

- Consistency
- Clarity
- Persistence
- Advocacy
- Compassion

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Practice Principles & Reminders

A guide for consistent, family-centered social work practice

Important Practice Reminder	**The system may be complex—** But your role is to make it understandable, manageable, and navigable for families.
Simple Takeaway	**Strong practice is not about doing more—** It is about doing the **right things consistently.**
Practice Reflection	**Ask yourself:** □ **Am** I anticipating barriers or reacting to them? □ **Is** my communication clear and consistent? □ **Am** I documenting and tracking effectively? □ **Are** families truly understanding the process? □ **Am** I advocating when systems fall short?
Final Thought	**When best practices are applied consistently:** ✓ **Families feel supported** ✓ **Systems become more navigable** ✓ **Outcomes improve** Your approach directly shapes the experience families have within the system.

Reflect → Anticipate → Communicate → Advocate → Deliver

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Becoming an OPWDD Self-Direction Support Broker

(New York City/State)

Overview

A Support Broker within the New York State Office for People with Developmental Disabilities (OPWDD) assists individuals with intellectual and developmental disabilities (I/DD) in managing their Self-Direction services. This includes developing individualized budgets, coordinating supports, and ensuring compliance with Medicaid and OPWDD regulations.

The Important Role of a Self-Direction Broker

The role of an SD can be both professionally fulfilling and financially lucrative, allowing practitioners to work closely with individuals and families to design and implement highly personalized support plans. At its core, support brokerage blends advocacy, coordination, and creativity, making the work not only impactful but also engaging and dynamic. However, success in this role requires a high level of attention to detail, as brokers must navigate complex systems, budgets, and compliance requirements. Equally important is a deep sense of compassion and commitment, as brokers play a critical role in helping families access resources, build meaningful supports, and enhance the quality of life for individuals with disabilities.

Step 1: Understand the Role and Requirements

Before beginning the process, it is essential to understand the core responsibilities of a Support Broker:

- Facilitate **person-centered planning**
- Assist with **budget development and resource allocation**
- Support individuals in **hiring and managing staff**
- Coordinate with the individual's **Circle of Support**
- Ensure adherence to **OPWDD and Medicaid compliance standards**

Reference:
OPWDD Self-Direction Overview
https://opwdd.ny.gov/types-services/self-direction

Step 2: Enroll in Required OPWDD Broker Training

Completion of OPWDD-approved training is mandatory.

Required Training Components:

- Self-Advocacy and Self-Determination
- Person-Centered Planning
- Broker Training Institute (four modules)
- Self-Direction Budgeting

Registration:

- Email: **SDBroker@opwdd.ny.gov**

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Training Information:

- https://opwdd.ny.gov/providers/self-direction-providers
- https://opwdd.ny.gov/providers/training

Training is completed through the **Statewide Learning Management System (SLMS)**.

Step 3: Complete Training via SLMS

All required courses must be completed through the New York State SLMS platform.

- Track course progress and completion
- Obtain training certificates
- Ensure all modules are successfully finalized

Estimated Timeline: Several weeks, depending on pacing and course availability

Step 4: Submit Support Broker Authorization Application

After completing training, you must apply for official authorization.

Required Submission:

- Support Broker Authorization Form
- Documentation verifying completed training

Submission Contact:

- Email: **SDBroker@opwdd.ny.gov**

Application Link:
https://opwdd.ny.gov/providers/support-broker-authorization-form

Step 5: Obtain OPWDD Approval

Upon approval:

- You will be added to the **OPWDD Support Broker Registry**
- You will be authorized to provide **billable Support Broker services**

Step 6: Determine Employment Structure

Option A: Employment Through an Agency

You may work under an OPWDD-approved provider agency (e.g., The Arc, QSAC, or other Self-Direction agencies).

Advantages:

- Administrative and billing support provided
- Structured onboarding and supervision

Option B: Independent Support Broker

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You may operate independently and contract directly with individuals and families.

Considerations:

- Must coordinate with a **Fiscal Intermediary (FI)**
- Responsible for maintaining compliance and documentation standards

Step 7: Establish Relationship with a Fiscal Intermediary (FI)

Support Brokers must work in coordination with an FI.

Role of Fiscal Intermediary:

- Process payroll for staff hired by individuals
- Manage billing and Medicaid claims
- Ensure regulatory compliance

Example Resource:
https://www.acds.org/programs-and-services/self-direction-fiscal-intermediary-and-brokerage-services

Step 8: Begin Providing Services

Once authorized, you may begin working with individuals enrolled in Self-Direction.

Referrals may come from:

- Care Managers
- OPWDD provider agencies
- Direct family inquiries

Step 9: Maintain Annual Authorization

Support Brokers are required to maintain active status through:

- Annual reauthorization
- Ongoing professional development and training compliance

Key Eligibility and Compliance Considerations

- Immediate family members or individuals residing in the same household **may not serve as paid Support Brokers**
- All required training must be completed prior to service delivery
- Strict adherence to OPWDD and Medicaid regulations is required

Core Competencies for Effective Practice & Person-centered planning and facilitation

- Budgeting and financial oversight
- Knowledge of disability services and systems
- Strong communication and advocacy skills
- Organizational and documentation proficiency

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Key Contacts and Resources
OPWDD Broker Training and Authorization
Email: **SDBroker@opwdd.ny.gov**

Official OPWDD Resources

- Self-Direction Overview: https://opwdd.ny.gov/types-services/self-direction
- Provider and Broker Information: https://opwdd.ny.gov/providers/self-direction-providers
- Training (SLMS): https://opwdd.ny.gov/providers/training

Compensation and Earning Potential for Support Brokers

Compensation for OPWDD Self-Direction Support Brokers in New York varies based on several factors, including whether you work for an agency or operate independently, your caseload size, and your level of experience.

Hourly Rates

- Entry-level or agency-based roles typically range from **$17 to $25 per hour**
- Mid-range positions often fall between **$21 to $30 per hour**
- Some experienced or specialized brokers may earn **$25 to $65 per hour**, particularly in independent or contract roles

Annual Salary Estimates

- Average salary in New York City: approximately **$88,000–$93,000 per year**
- National average: approximately **$72,000 per year**
- Entry-level or agency-based roles may range from:
  - **$37,000 – $55,000 annually**

Independent Broker Earnings (Important & Recommended)
For independent brokers, income is highly flexible and often more lucrative:

- Paid through Medicaid-funded Self-Direction budgets
- Earnings depend on:
  - Number of individuals served (caseload)
  - Complexity of cases
  - Hours billed per month

Many independent brokers:

- Start part-time and scale up
- Combine multiple cases to reach a full-time income
- Can exceed traditional salaries with a strong caseload

Key Factors That Influence Income

- Caseload size (more participants = higher income)

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- Experience and reputation in the field
- Ability to manage complex budgets and cases
- Whether you work independently vs. agency-employed
- Geographic region (NYC tends to pay higher)

Professional Insight
This role offers ***significant earning flexibility*** for professionals seeking:

- Autonomy
- Flexible scheduling
- Opportunities to scale income

Support brokerage can be a ***strategic and financially rewarding career pathway,*** especially when transitioning into independent practice.

Compensation Per Case: Monthly Earnings for Support Brokers

Understanding how Support Brokers are compensated is essential for evaluating the financial viability of this role. In New York State, Support Brokers are reimbursed through Medicaid under the OPWDD Self-Direction model, with services billed in **15-minute increments (units)**.

Medicaid Billing Rate

- Approximately **$11.57 per 15-minute unit**
- Equivalent to approximately **$46 per hour**, depending on billing and approval limits

Typical Monthly Earnings Per Individual

Monthly income varies based on the level of engagement required for each individual. The following ranges reflect common patterns in practice:

Low Engagement (Maintenance Phase)

- **Estimated Time:** 2–4 hours per month
- **Monthly Earnings:** Approximately **$90 – $185 per individual**
- Typically involves routine check-ins, minor adjustments, and documentation

Moderate Engagement (Standard Ongoing Support)

- **Estimated Time:** 5–10 hours per month
- **Monthly Earnings:** Approximately **$230 – $460 per individual**
- This is the most typical range for individuals with stable Self-Direction plans

High Engagement (Start-Up or Complex Cases)

- **Estimated Time:** 10–20+ hours per month
- **Monthly Earnings:** Approximately **$460 – $900+ per individual**
- Common during:
    - Initial plan development
    - Budget creation and revisions
    - Crisis intervention or complex care coordination

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Caseload-Based Income Projections

Because Support Brokers are paid based on billable time, overall income is directly tied to caseload size and engagement level.

Part-Time Caseload

- 10 individuals × ~$300/month
- **Estimated Income:** ~$3,000/month (~$36,000/year)

Full-Time Caseload

- 25 individuals × ~$300/month
- **Estimated Income:** ~$7,500/month (~$90,000/year)

Expanded Independent Caseload

- 30–40 individuals × $350–$500/month
- **Estimated Income:** ~$10,000 – $15,000+/month
- **Estimated Annual Income:** ~$120,000 – $180,000+

Key Considerations

- Compensation is ***time-based,*** not a flat rate per individual
- Monthly income may fluctuate depending on service needs
- Higher earnings are often associated with:
  - Start-up or complex cases
  - Larger caseloads
  - Independent practice models
- All billable time must be supported by **accurate and compliant documentation**

Professional Insight

Support Brokerage offers a level of ***income flexibility and scalability*** that is not typical in many traditional social work roles. Practitioners can increase earnings by expanding their caseload and managing their time efficiently. However, this model requires:

- Strong organizational and time management skills
- Careful attention to billing and compliance requirements
- Consistent engagement with individuals and families

Summary

- Typical monthly earnings per **individual – per case**: **$200 – $500 per month**
- Higher-need cases may temporarily exceed **$800 – $1,000 per month**
- Annual income potential varies widely based on caseload and structure, with experienced independent brokers often exceeding traditional salary ranges

DECISION TREE: RESPONDING TO SYSTEM BARRIERS

What to Do When Services Stall or Problems Arise

START HERE

What is the main issue?

1 COMMUNICATION DELAYS

No response, unanswered emails/calls

Step 1
Follow up (call + email) within 3–5 days

Step 2
Document attempt (date, name, outcome)

Did you receive a response?

- **YES** → Continue coordination
- **NO** → **Step 3**: Follow up again within 5–7 days

Still no response?

Contact supervisor or agency main line
Escalate concern

2 PROVIDER SHORTAGE

"No staff available" / waitlists

Step 1
Request additional provider options

Step 2
Contact multiple agencies directly

Is a provider available?

- **YES** → Begin intake process
- **NO** → **Step 3**:
  - Expand geographic search
  - Explore alternative services

Still no services?

Notify Care Manager
Request interim supports

3 ELIGIBILITY DELAYS

Application stalled, no updates

Step 1
Confirm all documents were submitted

Step 2
Contact DDRO for status update

Is anything missing?

- **YES** → Submit immediately
- **NO** → **Step 3**: Request timeline

Delay continues?

Escalate to supervisor
Increase follow-up frequency

4 POOR CARE MANAGEMENT

Lack of communication, no progress

Step 1
Contact Care Manager directly

Step 2
Request update or meeting

Improvement?

- **YES** → Continue monitoring
- **NO** → **Step 3**: Contact CMO

Still unresolved?

Request supervisor involvement
Request new Care Manager

5 PARENT BURNOUT

Exhaustion, frustration, withdrawal

Step 1
Acknowledge and normalize feelings

Step 2
Assess immediate needs

Step 3
- Connect to respite services
- Suggest support networks

Is stress improving?

- **YES** → Continue support
- **NO** → Increase check-ins + referrals

QUICK RULE ACROSS ALL BARRIERS

Follow up again → Document everything → Escalate when needed

CORE DECISION FRAMEWORK

When something goes wrong, always ask:

1. What is the barrier?
2. What action has already been taken?
3. What is the next level of follow-up?
4. Who needs to be involved next?

BEST PRACTICE REMINDER

- Do not wait for systems to resolve issues on their own
- Do not assume someone else is following up
- Do not stop at one attempt

PERSISTENCE MOVES CASES FORWARD

SIMPLE TAKEAWAY

Identify
Act
Follow up
Escalate

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Dear Social Worker,

It is with deep appreciation and genuine admiration that I close this handbook with a direct message to you. The work you do every day, navigating complex systems, holding space for families in crisis, and advocating when no one else will- is not invisible. **It is essential.**

As a social worker, you are not just navigating systems; you are helping families move through some of the most **complex and emotionally demanding** experiences of their lives. The weight of that responsibility is real, and so is the impact you carry.

So I want to say this:

Your Role Matters
Every interaction shapes a family's experience through the system.

Your Voice Matters
You amplify voices that might otherwise go unheard.

Your Advocacy Changes Outcomes
Compassionate persistence keeps families from falling through the cracks.

These are not just professional obligations; they are the foundation of trust that families place in you when they have nowhere else to turn. That trust is a privilege, and your practice is the answer to it.

> **As you carry this work forward, remember:**
> - Show up with intention
> - Listen before you lead
> - Document with purpose
> - Advocate without hesitation
> - Follow up with consistency

When best practices are applied consistently, when families feel seen, when services are delivered, and when systems are held accountable, **outcomes change. Lives change.** And at the center of that change is you.

Thank you for your dedication, your persistence, and your humanity. The families you serve are better for it.

With respect and gratitude,

Dr. Sabretta Alford

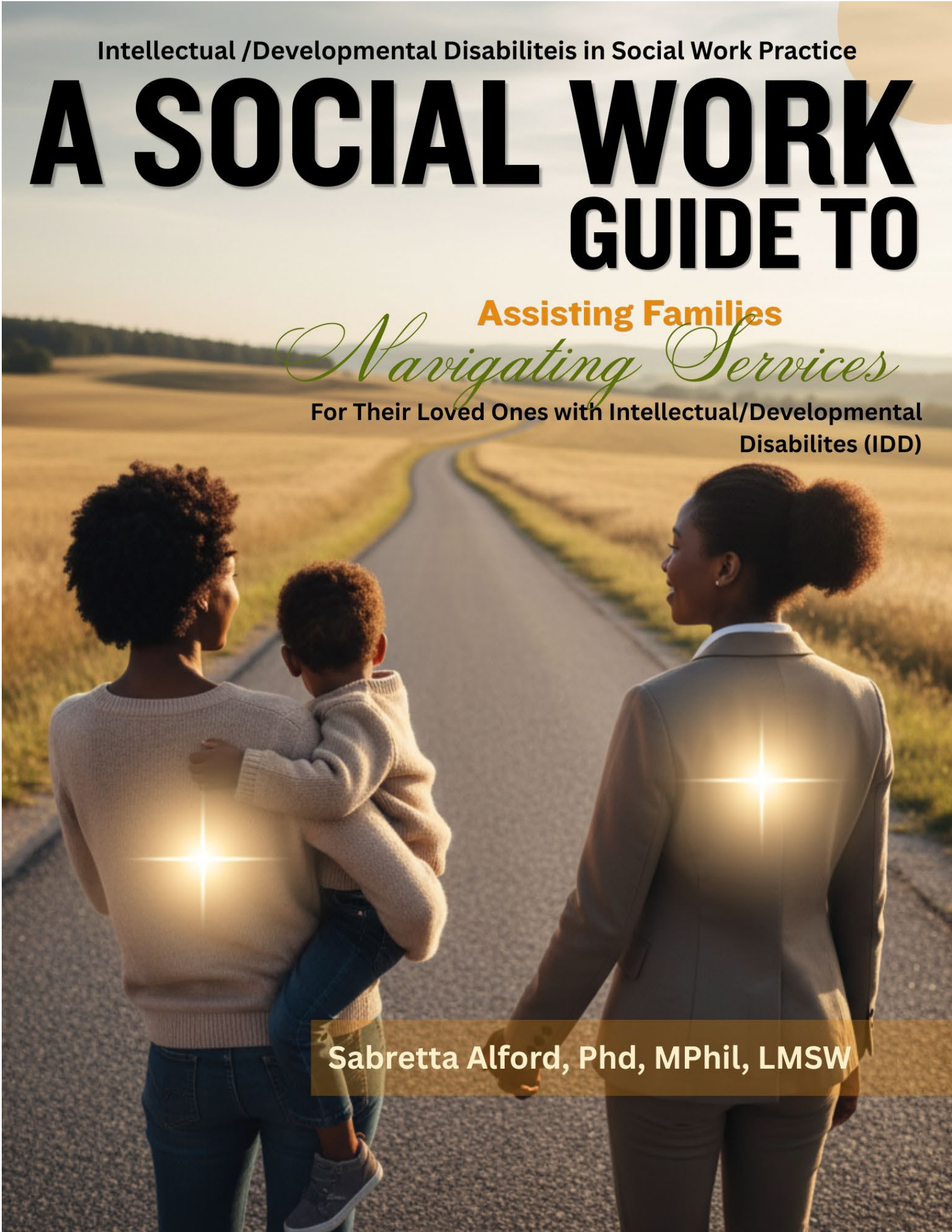
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Intellectual /Developmental Disabiliteis in Social Work Practice

A SOCIAL WORK GUIDE TO

Assisting Families *Navigating Services*

For Their Loved Ones with Intellectual/Developmental Disabilites (IDD)



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